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PATHOLOGY LABORATORY

Excellence in Diagnostics

Plot No. 29 D-7, Apeksha Pujan CHS, RSC-20, Gorai-1, Borivali (W), Mumbai - 400 092.

Patient Id : SPL24/1988
Patient : MR NEEL PATEL
Age/sex : 20 Yrs/Male
Ref. By : Dr. J.K.RATHOD
Center : SHREE MEDICAL CENTRE

Sample Id : 2408024
Registration Date : 02/08/2024 12:04 PM
Report Date : 02/08/2024 01:16 PM



HAEMATOLOGY REPORT

Test Description	Result	Unit	Biological Reference Range
RBC PARAMETERS			
Haemoglobin	8.90	gm/dL	13 - 17
RBC Count	2.29	$\times 10^6/\mu\text{L}$	4.5 - 5.5
HCT	26.10	%	40 - 50
MCV	113.97	fL	80 - 100
MCH	38.86	pg	27 - 32
MCHC	34.10	g/dl	31.5 - 34.5
RDW	21.20	%	11.6 - 14.0
WBC PARAMETERS			
Total WBC Count	7360	cell/cu.mm	4000 - 10000
Neutrophils	57	%	40 - 80
Lymphocytes	35	%	20 - 40
Eosinophils	03	%	00 - 06
Monocytes	05	%	2 - 10
Basophils	00	%	0 - 2
Band Forms	00	%	0 - 0
Absolute Differential Count			
Absolute Neutrophils Count	4195	/cumm	2000 - 7000
Absolute Lymphocyte Count	2576	/cumm	1000 - 3000
Absolute Eosinophil Count	221	/cumm	20 - 500
Absolute Monocyte Count	368	/cumm	200 - 1000
PLATELET PARAMETERS			
PDW	16.40	%	11.0 - 18.0
Platelet Count	174000	/cumm	150000 - 450000
MPV	9.10	fL	6.0 - 11.0
Peripheral Blood Smear	Hypochromia (+) Microcytosis (+) & Anisocytosis		
RBC Morphology	Normal		
WBC MORPHOLOGY	Adequate On Smear		
PLATELET MORPHOLOGY			

NOTE

- As per the recommendation of International council for Standardization in Hematology, the differential leukocyte counts are additionally being reported as absolute numbers of each cell in per unit volume of blood
- Test conducted on EDTA whole blood

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M.D.(Pathology)



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BIOCHEMISTRY REPORT

Test Description	Result	Unit	Biological Reference Range
LIVER FUNCTION TEST (LFT)			
TOTAL BILIRUBIN <i>Method: Serum, Jendrassik Grof</i>	2.89	mg/dl	0.3 - 1.2
DIRECT BILIRUBIN <i>Method: Serum, Diazotization</i>	0.42	mg/dL	0 - 0.3
INDIRECT BILIRUBIN <i>Method: Serum, Calculated</i>	2.47	mg/dl	0.1 - 1.0
SGOT (AST) <i>Method: Serum, UV with P5P, IFCC 37 degree</i>	28	U/L	8 - 45
SGPT (ALT) <i>Method: Serum, UV with P5P, IFCC 37 degree</i>	31	U/L	3 - 41
ALKALINE PHOSPHATASE <i>Method: DGKC</i>	63.0	U/L	35 - 147
TOTAL PROTEIN <i>Method: Serum, Biuret, reagent blank end point</i>	7.99	g/dl	6.6 - 8.3
SERUM ALBUMIN <i>Method: Serum, Bromocresol green</i>	4.88	g/dl	3.5 - 5.5
SERUM GLOBULIN <i>Method: Serum, Calculated</i>	3.11	g/dl	1.8 - 3.6
A/G RATIO <i>Method: Serum, Calculated</i>	1.57		1.2 - 2.2

NOTE :

In known cases of Chronic Liver disease due to Viral Hepatitis B & C, Alcoholic liver disease or Non alcoholic fatty liver disease, Enhanced liver fibrosis (ELF) test may be used to evaluate liver fibrosis.

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SEROLOGY & IMMUNOLOGY REPORT

Test Description	Result	Unit	Biological Reference Range
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WIDAL TEST

S.Typhi "O" Antigen	1:80
S.Typhi "H" Antigen	1:40
S.ParaTyphi "AH" Antigen	Negative
S.ParaTyphi "BH" Antigen	Negative

Method: Slide Agglutination Method

The Widal test is positive if TO antigen titer is more than 1:160 in an active infection, or if TH antigen titer is more than 1:160 in past infection or in immunized persons. A single Widal test is of little clinical relevance due to the high number of cross-reacting infections, including malaria. If no other tests (either bacteriologic culture or more specific serology) are available, a fourfold increase in the titer (e.g., from 1:40 to 1:640) in the course of the infection, or a conversion from an IgM reaction to an IgG reaction of at least the same titer, would be consistent with a typhoid infection.

CRP (C-REACTIVE PROTEIN)	0.7	mg/L	0 - 6
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Method: Immunoturbidometry

Test Usage:

Determine the presence of C-reactive protein. Nonspecific, but most sensitive indicator of inflammation and/or necrosis. A baseline presurgical CRP may be useful in determining postoperative infection.

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URINE EXAMINATION REPORT

Test Description	Result	Unit	Biological Reference Range
URINE ROUTINE			
Physical Examination			
Quantity	20ml	ml	PALE YELLOW
Colour	Pale Yellow		Slightly Hazy
Apperance	Slightly Hazy		Alkaline
Reaction(pH)	6.0		
Specific Gravity	1.015		
Chemical Examination			
Protein	Trace		Absent
Sugar	Absent		Absent
Ketone Bodies	Absent		Absent
Bile Salt	Absent		Absent
Bile Pigment	Absent		Absent
Occult Blood	Absent		Absent
Microscopic Examination			
Pus cells	4-6/hpf	/hpf	1-2/hpf
RBC's	Absent	/hpf	Absent
Epithelial Cells	2-4/hpf	/hpf	1-2/hpf
Amorphous Deposit	Absent		Absent
Mucus Threads	Absent		Absent
Candida	Absent		Absent
Bacteria	Absent		Absent
Yeast Cells	Absent		Absent
Casts	Absent		Absent
Crystals	Absent		Absent
Other Finding			
Note	Sample quantity less than 12 ml.,		

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Center : SHREE MEDICAL CENTRE



SEROLOGY & IMMUNOLOGY REPORT

Test Description	Result	Unit	Biological Reference Range
ANTI HCV	NON REACTIVE		< NON REACTIVE >

Method : RAPID

Principle :

Test detects antibodies to antigens NS3, NS4ab, NS5core and chimeric protein NC34ab.

Test Limitations :

Window period of 6-9 months may be seen after infection and appearance of antibodies. Occasional cross reactions with CMV, tuberculosis, HSV, Rubella, Syphilis may be seen.

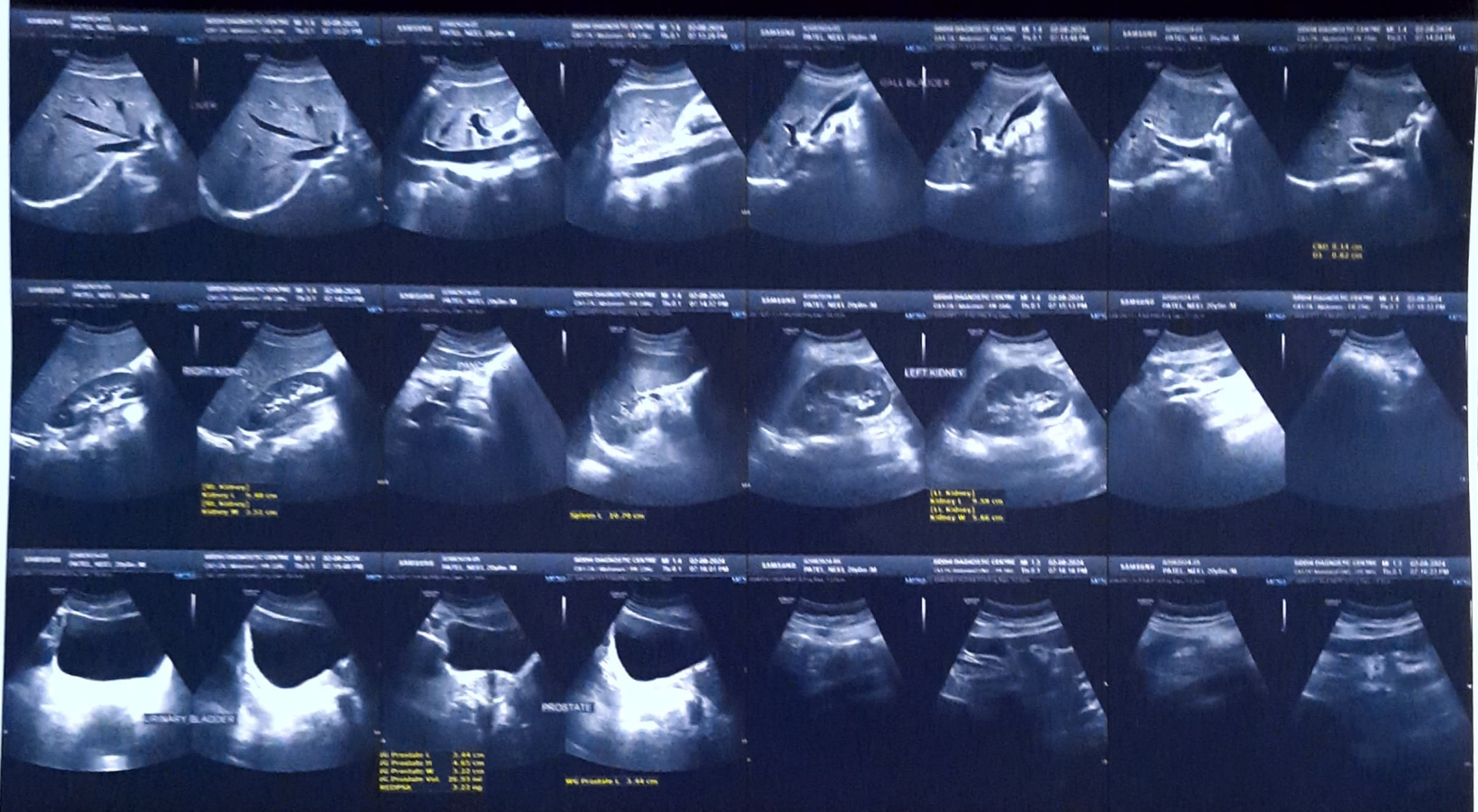
HBsAg -Hepatitis B surface antigen NON REACTIVE

Clinical significance:

Hepatitis B Surface Antigen with Reflex Confirmation - Surface antigen usually appears in the serum after an incubation period of 1 to 6 months following exposure to Hepatitis B virus and peaks shortly after onset of symptoms. It typically disappears within 1 to 3 months. Persistence of Hepatitis B surface antigen for greater than 6 months is a prognostic indicator of chronic Hepatitis B infection.

-----End Of Report-----

DR.SIDDHARTH YADAV
M.D.(Pathology)





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SIDDHI DIAGNOSTIC CENTRE

Sumil Co-op Hsg Soc., 1st Floor, Sector No. 5, Charkop, Kandivali (West), Mumbai - 400 067. Tel: 2869 1588, 7304647998

Patient's Name :	Mr. NEEL PATEL	Age : 20 Years	Date : 02-Aug-24
Ref. by :	Dr. J K RATHOD		
Examination :	ABDOMEN PELVIS		

Real time, B mode, gray scale sonography of the abdominopelvic organs was performed.

LIVER : It is normal in size, shape and has smooth margins. It is uniformly isoechoic. The portobiliary radicles are normal in distribution and calibre. There is no solid or cystic mass lesion or calcification. PORTAL VEIN is normal in calibre.

GALL BLADDER : is well distended and has uniformly thin and regular walls. There is no mass lesion or calculus.

COMMON BILE DUCT : is normal in calibre 0.14 cm.

PANCREAS : are normal in size, shape, contour and echotexture. No solid or cystic mass lesion is noted.

KIDNEYS : Right kidney measures 9.48 cm x 3.51 cm, Left kidney measures 9.59 cm x 5.66 cm.. Both the kidneys are normal in size and have smooth margins. Cortical echotexture is normal. No calculus or hydronephrosis is seen.

SPLEEN : is normal in size 10.70 cm and shape. The echotexture is homogenous without any focal lesion.

URINARY BLADDER

The urinary bladder is well distended. The walls are normal in thickness. There is no calculus. No mass or diverticulum is seen.

PROSTATE : The prostate gland shows well defined and smooth margins. It reveals normal echotexture. No focal lesion is seen. No calcification is seen. It measures 3.22 cm x 4.65 cm x 3.44 cm, It weight is 26.93 ml gms.

There is no free fluid.

RIGHT ILIAC FOSSA :- No evidence of bowel wall thickening or dilated bowel loops. No evidence probe tenderness or free fluid is seen.

CONCLUSION : No significant abnormality is detected in the sonographic evaluation of the abdomino-pelvic organs.

SUGGEST : clinical correlation

Dr. Kshitij Pawar
Radiologist